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SYSTEMATIC REVIEW

Epidemiology of falls in community-dwelling older adults in Europe: a systematic review and meta-analysis

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Abstract

Objectives: Falls have long been recognised as a frequent problem among older adults and have been cited in literature since the 1950s. Given extensive research on risk factors, prevention, and implementation strategies, one might expect a decline in fallers prevalence. The aim of this review is to explore the epidemiology of falls in Europe, focusing on healthy, community-dwelling individuals aged 65 years or older.
Methods: Articles for this systematic review and meta-analysis were sourced from PubMed and Web of Science in June 2023, with screening completed by August 2023 and an update in January 2024. Risk of bias assessment used the Standard quality assessment criteria and potential outliers were identified. Publication bias was assessed using Egger's regression test. Data analysis was performed in R.
Results: Thirty-eight articles were included, comprising a sample of 71 245 European, community-dwelling older adults. The average fallers prevalence among European older adults was 30% (95% CI 0.26–0.34). Meta-regression analysis showed no significant change in fallers prevalence over the years ($P = .66$), in contrast with meta-regression for average age ($P < .01$). In the subgroup analysis, differences in fallers prevalence were seen for gender ($P < .01$), country ($P < .01$), and length of follow-up ($P = .02$).
Conclusion: Despite decades of evidence supporting effective fall prevention, there is no significant change in the prevalence of fallers among community-dwelling older adults in Europe. Future research should focus on systematically identifying the factors contributing to the persistent fall rates. Additionally, efforts must be made to ensure effective implementation of existing knowledge on fall prevention.

Keywords: older adults; falls; epidemiology; prevalence; meta-analysis; systematic review; older people

Key Points

- The average fall prevalence among European, community-dwelling older adults is 31%.
- Analysis indicated no significant differences in fall prevalence over the years, suggesting no change over the past decades.
- The lack of decline in fall rates contrasts with the vast amount of research that has been done regarding fall prevention.
- In the future, efforts must be made to ensure that existing knowledge on fall prevention is effectively implemented.

Introduction

Life expectancy in Europe has significantly increased over the past decades, reaching 83.3 years in the European Union. This drives the continuing trend that there are more older adults than ever before [1, 2]. In 2019, over a billion people worldwide were aged 60 years or older. By 2050, that number is predicted to double [2]. Although life expectancy has increased due to advancements in healthcare, healthy life expectancy has not increased at the same rate [3]. Healthy life expectancy considers how long a person can live in full health, accounting for years affected by disease or injury. There are, in fact, several unfavourable health-related effects associated with ageing. Falls are a well-known and common issue among older adults. The first articles on falls among community-dwelling older adults (living at their own home) were already published in the 1950s [4, 5]. The economic burden caused by falls has been recognised as early as 1989 [6]. Today, the amount of research on falls in older adults is extensive, ranging from studies on risk factors to investigations into prevention and implementation strategies.

The devastating impact of falls has been recognised and described in nearly every country in the world [1, 6]. Understanding the epidemiology of falls in community-dwelling older persons in Europe is crucial to inform preventive strategies and health policy [7]. For the past three decades, falls have been a significant public health concern, particularly among older adults, as they can lead to severe injuries, disability, reduced quality of life, and increased mortality. Many of these consequences are accompanied by visits to the emergency department or hospitalisation and can eventually lead to admission to a rehabilitation facility or institutionalisation in long-term care facilities. In addition to the personal burden for patients, the consequences of falls have also an economic impact [1]. The costs for health and social care services per fall are substantial [8]. Since falls are a frequently cited problem in the ageing population, the annual costs are expected to be extensive and with the growing population in mind, these costs will continue to rise.

To reduce the number of falls, an accurate screening tool to look for older adults who may be at risk for falls is an essential starting point. In the past decades, many tools have been developed and examined on their ability to find these older persons at risk, for example, the Timed up and Go and the Single leg stance [9, 10]. Following studies evaluating single screening tools, numerous systematic reviews have been carried out on this subject to determine a gold standard [11, 12]. Unfortunately, no single test has proven feasible to use in isolation to predict fall risk. A comprehensive evaluation including multiple assessments of gait, balance and strength, medical history and self-reported assessments is recommended [11, 13].

In addition to research on screening tools, there is also an extensive amount of information regarding prevention and treatment options. Multifactorial interventions are identified as the most effective methods for reducing the number of falls [14]. However, these interventions are time-consuming and therefore primarily recommended in the 'high-risk' population [15]. In such interventions, participants are evaluated for multiple risk factors and receive a personalised intervention programme to address their personal risks, which creates a safer environment [7]. For the 'average-risk' population, exercises are considered essential, focusing on improving balance, increasing muscle strength and ameliorating mobility [15–17]. An important next step after developing intervention programmes is implementing them in real-life settings among the ageing population. Research suggests that the most effective strategy for reducing falls is to deliver physically engaging, multifactorial interventions through interprofessional collaboration [18, 19]. Effective implementation is crucial for achieving a successful outcome in older adults [20].

As mentioned earlier, falls in older adults have been extensively researched, resulting in a wealth of information about fall risks, falls, and evidence-based treatment options. Given this vast body of knowledge, one might expect a significant decline in the prevalence of falls reported in research. However, it is assumed that this may not be the case. In 1948, the book *The Social Medicine of Old Age* mentioned that slightly over one-third of older adults experienced falls [21]. Similarly, in 1995, an article reported that approximately one-third of the older adults (over 65 years old), living in the community, fall each year [22]. In 2007, a World Health Organisation (WHO) report stated that 28%–35% of older adults fall each year and in 2022 a systematic review reported a fallers prevalence of 26.5% worldwide [1, 23]. The fallers prevalence has remained consistent over time, which contrasts with the initial thought that anticipated a decline in fallers prevalence. To further elaborate on this topic, the purpose of this review is to explore the epidemiology of falls in Europe, focusing on healthy, community-dwelling individuals aged 65 years or older. The review aims to highlight the evolution and status of this long existing, but still persistent issue.

Methods

This review was conducted in accordance with the Preferred Reporting Items for Systematic reviews and Meta-Analysis guidelines and was registered with PROSPERO (ID: CRD42024503046) [24].

Search strategy and selection criteria

The screening was conducted between 26 June and 14 August 2023. The databases used for extracting articles were Web of Science (WoS) and PubMed. The used search strategies can be found in Supplementary Appendix 1. To identify potential additional articles missed by these search strategies, the references of relevant systematic reviews regarding fallers prevalence and incidence in older adults were screened by title and abstract. From this hand search, additional 12 articles were screened in full text. In January

Table 1. The in- and exclusion criteria used during the entire screening process.

	Inclusion	Exclusion
P	Community-dwelling older adults Healthy older adults Age > 65 years European	Institutionalised Age < 65 years Sample size $n \leq 20$ Hospitalised Neurological diseases Dementia Major surgical procedure during the past year Animal In vitro High fall risk at baseline
I	/	Therapy Medication Follow-up >36 months
C	/	/
O	Fall(s) prevalence Fall(s) incidence Epidemiology	Mortality Fractures
S	Randomised Controlled Trials Observational studies: cross-sectional Case control Longitudinal studies	Systematic reviews Meta-analyses Case-reports Editorials, letters, or opinion pieces
Language	Dutch English	Other languages

P: Population, I: Intervention, C: Comparison, O: Outcome, S: Study design.

2024, a screening update was conducted, but no new articles were added.

This review considered articles about fallers prevalence in European older adults (aged 65 years or older). The included population should be a representation of all the community-dwelling older adults. When only a small part (<10%) of the population met one of the exclusion criteria (P), the article was still included in this review. An article was excluded when the included population solely represented a group suffering from a particular disease. For prospective randomised clinical trials, only the data of the control group was considered. For retrospective randomised clinical trials, where fall history on baseline was known, data of all participants were included. Table 1 provides an overview of the inclusion and exclusion criteria.

Study selection

First, all articles were collected in Endnote, where duplicates were removed. All articles were imported into Rayyan, where they were screened by title and abstract [25]. Subsequently, the remaining articles were screened in full text. The exclusion sequence used for both screenings was: (i) duplicate, (ii) wrong language, (iii) wrong study design, (iv) wrong population, and (v) wrong intervention or outcome (Table 1). When the full text was not available online, it was requested from the authors. Uncertainty about a study's inclusion was resolved by consulting the co-author and PI (D.C.).

Data extraction

When a study specified a date for data collection, that year was assigned to the prevalence. If no date was mentioned, the publication year was used instead. For further analysis regarding the length of follow-up, the included studies are categorised into four subcategories: 3, 6, 12 and 24 months. In case of uncertainty regarding the data, the co-author and PI (D.C.) were contacted for deliberation.

Quality assessment

The 'Standard quality assessment criteria' (Supplementary Appendix 2) was used to evaluate and compare the risk on bias for studies with different study designs [26]. Articles were assessed on 14 distinct aspects, including research objectives, study design, sampling, random allocation and/or blinding, data collection, data analysis, confounding, verification, and reflexivity. Items not applicable to the study design were excluded from the total possible score. Studies were classified as having strong quality (final score of ≥0.80), good (final score of 0.71–0.79), adequate (final score of 0.50–0.70), or limited quality (final score of ≤0.5) [27].

Data synthesis and analysis

To detect outliers in this meta-analysis, upper and lower limits were calculated using the interquartile range (IQR) (Q1/Q3 +/− 1.5∗IQR). Publication bias was assessed using Egger's regression test by evaluating asymmetry in the funnel plot. Egger's test was performed on the subset of data that included the outcome of interest from the subgroup analysis [28].

A subgroup analysis was conducted to assess the significant differences in prevalence based on study design (retrospective or prospective), country, length of Follow-Up (FU), and gender, with the significance level set at 0.05

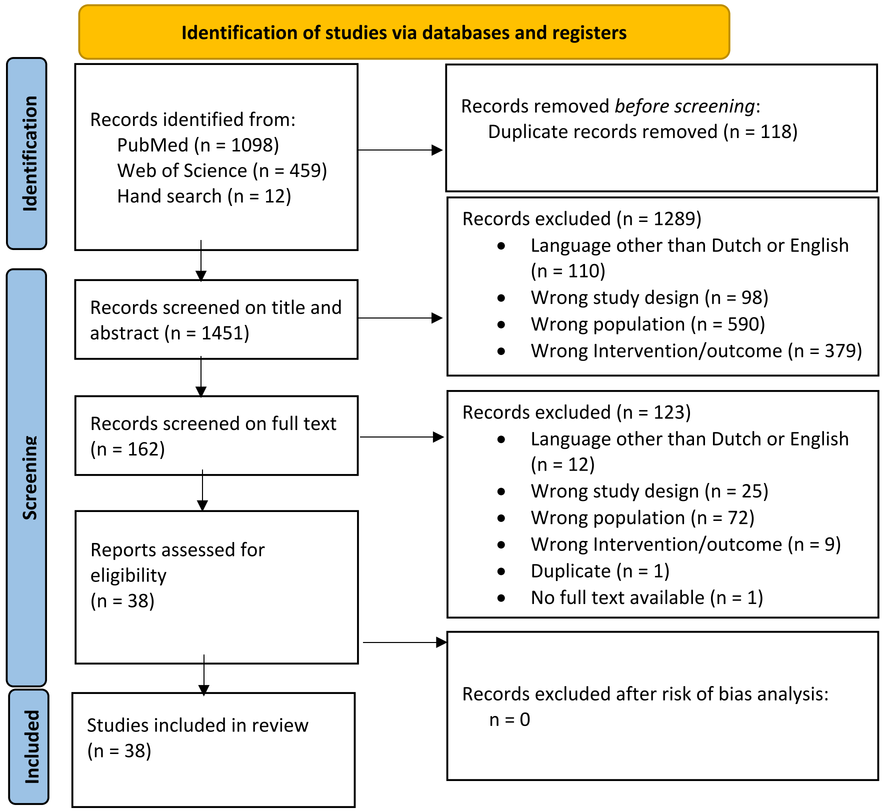

Figure 1. Flowchart of the screening process for including articles in this systematic review [24].

[29]. A meta-regression was performed to identify significant differences based on the year of data collection/publication and average age of the participants. Model building was performed by including significant subgroups into the meta-regression model and their interaction with the year of publication (year × subgroup) or their interaction with the average age of the participants (mean age × subgroup). To assess the interactions between the significant subgroups and the continuous variables (year and mean age), a P-value for the interaction terms of .017 (0.05 divided by 3, i.e. the number of separate interactions tested) was used to correct for multiple comparison and ensure a Family-wise error rate of 5%. Additionally, information on the moderator effect and AIC was assessed for each of the meta-regression models.

The final model was selected using a forward selection method, whereby only significant interactions were kept into the meta-regression model. Due to observed heterogeneity across studies, random effects models were used to describe the results. All statistical procedures were conducted using the 'meta' package in R, which provides robust methods for meta-analysis and meta-regression, effectively managing study variability and subgroup analyses.

Results

Figure 1 shows the flow diagram representing the in- and exclusion process. Finally, 38 articles were included in this review. Figure 1 shows the flow diagram representing the in- and exclusion process. The outcome of the risk of bias analysis is shown in Supplementary Appendix 3. No articles were excluded following the quality assessment. Study characteristics from the included studies are presented in Supplementary Appendix 4 and extracted data is presented in Table 2. Twenty-four studies described the used definition for a fall (Supplementary Appendix 5). No outliers were found.

The results showed significant differences in fallers prevalence based on gender, study design, country, and length of follow-up (Table 3, Supplementary Appendices 6–9). Conversely, meta-regression analysis did not find an association between the reported fallers prevalence among studies and the year of conduct, suggesting no change in the reported fallers prevalence over time (Beta: −0.0042 (SE = 0.0096) (95% CI [−0.0231, 0.0146]), $P = .6605$) (Supplementary Appendix 10). For mean age, a significant association was found, whereby a higher age was associated with a higher reported fallers prevalence (Beta: 0.061 (SE = 0.0202) (95% CI [0.0210, 0.1002]), $P = .0027$) (Supplementary Appendix 11). More details on the model selection steps can be found in Supplementary Appendix 12. Since the mean age by sex was only available in 4 studies, the interaction between age and sex was not evaluated.

The 38 articles that were included in this meta-analysis comprised a sample of 71 245 European, community-dwelling older adults. The average fallers prevalence is 30%

Table 2. Data extraction from the included articles in this systematic review.

Year	Country	Sample size (n)	Female (%)	Age related data (incl. criteria) (mean age (SD))	Number of fallers (n)	Prevalence of fallers (%)		
						M	F	Total
1974[a] [30]	United Kingdom	983	44.9	65+ /(/)	305	22.1	42.0	31.0
1977[a] [31]	United Kingdom	2357	58.9	65+ /(/)	661	19.0	34.4	28.0
1988 [32]	United Kingdom	1022	61.0	65+ /(/)	356	24.3	41.6	34.8
1988[a] [33]	United Kingdom	203	70.4	80+ 83.0 (5.0)	86	31.7	46.9	42.4
1991[a] [34]	United Kingdom	1774	/	75+ /(/)	214	/	/	12.1
1992[a] [35]	Italy	1288	57.0	65+ 74.2 (6.3)	178	10.3	16.5	13.8
1994 [36]	Finland	979	64.5	70+ /(/)	296	25.7	33.1	30.2
1994[a] [37]	France	7147	100	75+ 80.5 (3.8)	1886	/	26.4	26.4
1995[a] [38]	Norway	431	61.5	67+ /(/)	104	22.3	25.3	24.1
1998[a] [39]	Norway	300	100	75+ 80.9 (/)	151	/	50.3	50.3
1999[a] [40]	The Netherlands	1365	51.1	65+ 75.3 (6.4)	755	/	/	55.3
2000[a] [41]	Spain	448	59.2	65+ 74.6 (6.9)	144	25.1	37.0	32.1
2000 [42]	The Netherlands	1660	61.3	70+ /(/)	723	47.2	41.3	43.6
2000[a] [43]	The Netherlands	480	67.1	85 85 (/)	212	/	/	44.2
2001[a] [44]	Belgium	131	53.4	70+ 76.7 (5.4)	52	41.0	45.0	39.7
2001[a] [45]	Finland	555	77.3	85+ 88 (/)	273	43.7	50.8	49.2
2002 [46]	Italy	5570	58.9	/ 77.2 (/)	1997	35.1	36.3	35.9
2002 [47]	Norway	2441	54.1	65+ 72.0 (4.8)	803	32.2	33.5	32.9
2003 [48]	Finland	220	55.9	85+ 88 (3)	136	/	/	61.8
2004[a] [49]	Sweden	2865	55.7	60+ 69.3 (10.2)	330	9.2	13.3	11.5
2007 [50]	Denmark	94	74.4	70+ 73.7 (2.9)	14	/	/	14.9
2008[a] [51]	France	7643	50.5	65+ 70.9 (4.6)	1456	12.0	25.9	19.1
2008[a] [52]	Norway	6712	54.9	70+ /(/)	1344	17.5	22.1	20.0
2009 [53]	Spain	640	60.3	75+ 81.3 (5.0)	160	21.7	27.2	25.0
2010[a] [54]	France	96	78.1	65+ 82.0 (5.7)	48	/	/	50.0
2011 [55]	France	1759	51.0	65+ 70.7 (4.6)	563	21.9	41.7	32.0
2012 [56]	The Netherlands	2971	67.9	65+ 81.5 (7.0)	362	12.1	12.2	12.2
2012 [57]	Poland	4920	48.5	65+ 79.4 (8.7)	1138	19.6	26.8	23.1
2012[a] [58]	France	1023	50.7	65+ 70.5 (5.0)	303	21.2	37.8	29.6
2013[a] [59]	United Kingdom	7236	54.2	60+ 70.4 (/)	2038	25.9	30.1	28.2

(continued)

Table 2. Continued.

Year	Country	Sample size (*n*)	Female (%)	Age related data (incl. criteria) (mean age (SD))	Number of fallers (*n*)	Prevalence of fallers (%)		
						M	F	Total
2013[a] [60]	Spain	572	63.3	69+ 76.1 (3.9)	204	24.8	42.0	35.7
2015 [61]	Sweden	1877	48.7	70 70 (/)	255	12.0	15.2	13.6
2015[a] [62]	Norway	108	62.0	/ /(/)	45	/	/	41.6
2016[a] [63]	France	1471	67.0	/ 72.5 (5.1)	485	29.3	34.5	32.9
2016[a] [64]	Italy	1220	43.1	65+ /(/)	142	/	/	11.6
2019[a] [65]	Spain	498	67.1	65–80 71.2 (4.2)	148	/	/	29.7
2020 [66]	Italy	96	64.6	65+ 77.2 (6.5)	32	29.4	35.5	33.3
2020[a] [67]	Greece	90	85.6	65+ 72.6 (6.2)	44	/	/	48.9

Abbreviations: Retro: retrospective, Pro: prospective; mo: months; /: missing data. [a]Year the study was conducted (different than year of publication).

Table 3. Results meta-regression.

		Prevalence [95% CI]	***Q* (*df*)**	***P*-value**
Gender (*n* = 28)	- Women (*n* = 28)	32% [0.27; 0.37]	6.677 (1)	<.01
	- Men (*n* = 26)	23% [0.19; 0.28]		
Study design (*n* = 38)	- Prospective (*n* = 14)	35% [0.27; 0.44]	2.62 (1)	.11
	- Retrospective (*n* = 24)	27% [0.23; 0.32]		
Country (*n* = 38)	- United Kingdom (*n* = 6)	28% [0.21; 0.37]	218.61 (11)	<.01
	- Italy (*n* = 4)	21% [0.13; 0.34]		
	- Finland (*n* = 3)	47% [0.32; 0.62]		
	- Norway (*n* = 5)	33% [0.24; 0.43]		
	- The Netherlands (*n* = 4)	36% [0.20; 0.56]		
	- Spain (*n* = 4)	30% [0.27; 0.35]		
	- Belgium (*n* = 1)	40% [0.31; 0.49]		
	- Sweden (*n* = 2)	12% [0.11; 0.14]		
	- Denmark (*n* = 1)	15% [0.08; 0.24]		
	- France (*n* = 6)	30% [0.24; 0.37]		
	- Poland (*n* = 1)	23% [0.22; 0.24]		
	- Greece (*n* = 1)	49% [0.38; 0.60]		
Length of follow up (months) [37]	- 3 (*n* = 3)	18% [0.09; 0.32]	9.94 (3)	.02
	- 6 (*n* = 3)	20% [0.13; 0.29]		
	- 12 (*n* = 29)	32% [0.28; 0.37]		
	- 24 (*n* = 2)	0.41 [0.24; 0.61]		

(95% CI 0.26–0.34) (Figure 2). The funnel plot shows potential for publication bias (Supplementary Appendix 13); however, Egger's test ($t = 0.99$, $df = 36$, $P = .330$) suggests there is no significant evidence of publication bias in the meta-analysis. In the general analysis, the subgroup analysis for study design and country, as well as in the meta-regression for year of data collection/publication, all articles were included (Table 3). For gender analysis, only 26 articles provided information on falls for both men and women, while additional two reported data only for women. Twenty-eight articles provided information on average age and all articles reported the length of FU.

Discussion

General analysis

Several thoughts or explanations for the absence of a decline in fallers prevalence can be considered in view of the results of this systematic review. First, it is important to look at the proportion of women in the studies. Women have a higher risk on falls, so in case there would be extensively more women in the included studies, it could lead to a false representation of the overall European population. The proportion of women in all included studies is 60.4%. When excluding the studies that only include women, the percentage decreases to 55.6%. Studies up to the year 2000 exist of 56.8% women, where

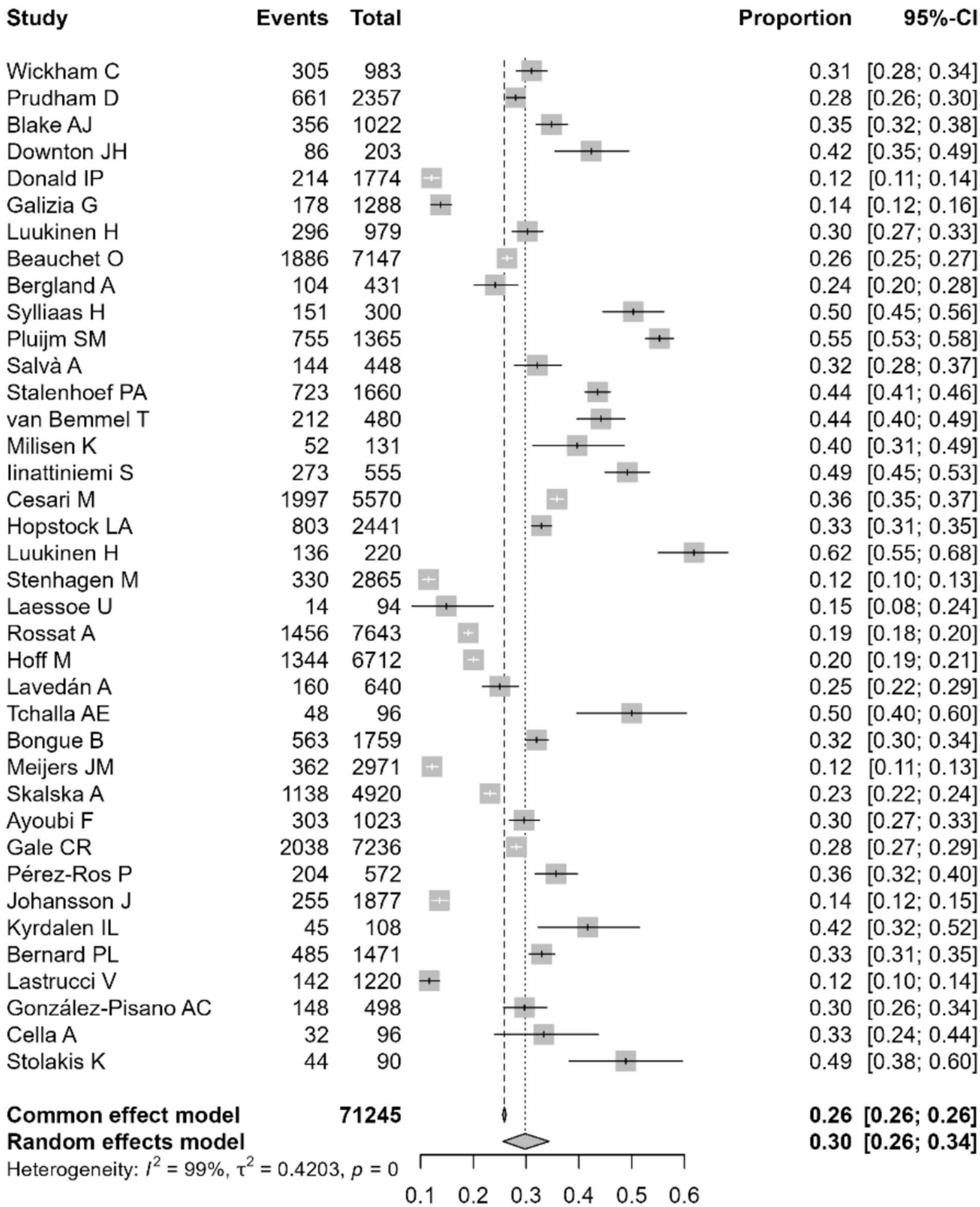

Figure 2. Forest plot for prevalence of falls in general.

studies from 2001 to 2010 and studies from 2011 onwards respectively exist of 55.8% and 54.9% women. The sex ratio in Europe has been equalising in the past decades and was 57.1% in 2019 [68]. The included studies thus reflect a good representation in terms of gender distribution in Europe and cannot directly be an explanatory reason for the absence of the decline.

A second consideration is the mean age in the studies since advanced age is a well-known risk factor for falls. An increasing mean age would also correctly reflect the increasing proportion of people aged 80+ in Europe and could contribute to a higher overall fallers prevalence [1]. However, when looking at this review, the mean age is decreasing through the years, which means that the included age from the sample also cannot be an explanation for the absence of decline (Supplementary Appendix 14). Some explanations came up when discussing this noteworthy display. Recent studies may have used electronic devices like email and social media, which may not reach all older adults, especially octogenarians. Furthermore, the sampling policies have changed in the past decades. In older included studies, the methods sometimes describe that all citizens of a particular town were approached for participation in their study. This way of recruitment is no longer allowed due to General Data Protection Regulation. Now, researchers need to approach the older adults in other ways, which makes it much more difficult to contact potential participants directly, especially the more isolated, oldest older adults.

Finally, it is possible the oldest studies may have underreported fallers prevalence due to methodological limitations. For instance, it is likely that follow-up methods have become more efficient and detailed over time and more recent cohorts of older persons have also become more digitally literate making people much more reachable in a variety of modes. However, almost all included studies questioned the fall history, and digital ways for follow-up were seldomly mentioned. So, the explanation for the absence of decline is still lacking.

Gender and age

As mentioned earlier, advanced age and female sex are well-known risk factors for falls [1, 69]. The fallers prevalence in women in this study was significantly higher than in men ($P < .01$), which aligns with the existing literature [1, 70]. The same statement can be made regarding the average age, where a higher age was associated with a higher reported fallers prevalence ($P = .0027$).

Retrospective vs prospective studies and length of follow-up

Most studies in this review had an FU of 3, 6 or 12 months. Six studies with follow-ups ranging from 1 to 36 months were grouped accordingly: 1 and 1.5 months with the 3-month group, 11 and 16 months with the 12-month group, and 24 and 36 months formed a separate 24-month group. As expected, faller prevalence was lowest in the 3-month group and highest in the 12- and 24-month groups due to the longer observation period. It should be considered that it is possible that longer follow-up periods in retrospective studies increase the risk of recall bias [71]. Older adults often forget to report falls when interviewed about those that occurred during the past 3 to 12 months [72]. Prevalence of fallers in retrospective studies may therefore be underreported. When looking at the prospective studies, it is important to keep the recall period into account. For instance, the study by Pluijm et al. [40] with a follow-up of 36 months had little risk of recall bias, since participants were asked to fill out fall calendars every week.

Region

The significant differences observed in the sub-analysis by region can be attributed to the major heterogeneity among the studies, driven by differences in included age and gender, study design, follow-up length and the limited number of studies representing each country. However, it cannot be definitively stated that there are no differences in fallers prevalence between the countries. More research using harmonised study protocols across countries is needed to draw correct conclusions.

Limitations

First, it should be noted that not all European countries provided usable data for this review. The data came from only 12 countries, and the oldest articles ($n = 5$ studies) (1974 until 1991) exclusively represented fallers prevalence in the UK. Secondly, differences in study populations may have introduced baseline bias. Besides age and gender variations, a small proportion of participants had exclusion criteria characteristics, such as Parkinson's disease or a history of stroke. While the impact of this is considered minor, it may have influenced the reported fallers prevalence.

Explanatory factors

The fact that the prevalence of fallers has not declined in recent decades and the explanation for this cannot be found in the results, raises questions about other possible explanatory factors. For example, globalisation and urbanisation have led to more older adults living at home without or with less support from their families, which increases the risk of falling [7]. Following this, some older adults may deny their fall risk out of fear that acknowledging it could lead to changes in how family and society perceive them or result in suggestions that may compromise their independence. In contrast, others may be unaware of their risk or see falling as a normal part of ageing [73]. It is essential to provide accurate information about falls and fall prevention, clearly outlining the general approach and eligibility criteria for fall prevention programmes [74].

Another explanation is the lack of an accurate fall screening tool that effectively identifies older adults who could benefit from prevention. Future efforts should focus on improving screening methods and refining cut-off values to better detect varying levels of fall risk. Current tools may miss those with modifiable risk and only catch individuals at a stage where interventions have a limited effect. A more nuanced approach could help to reach high-risk individuals and intervene earlier in those with manageable risks.

Besides the lack of accurate screening tools, there may be a significant gap between the detailed description of research-based prevention strategies and their effective implementation in real life [20]. Successful programmes require adaptation from both participants and their environment [73]. Due to the complexity of real-world settings, implementing multifactorial strategies is challenging and often less effective than intended [75, 76]. The challenge remains in effectively implementing these approaches in practice [77]. Several factors need to be considered, such as time, funding, and staffing. For instance, a skilled physiotherapist may need to acquire additional training to deliver evidence-based fall-prevention treatments [77]. Training, coaching and performance assessment for these professionals may be necessary to effect meaningful change at the practice level. Introducing recognition or certification for health care services and fall-prone older adults, could help ensure that specifically trained physiotherapists are engaged in fall prevention efforts. Additionally, respecting older adults' wishes and needs is crucial [75]. Increased multidisciplinary consultations can help address all relevant needs and ensure comprehensive care [20].

Conclusion

This systematic review and meta-analysis reveal that, despite decades of evidence supporting effective fall prevention, there are no significant changes in the prevalence of falls among community-dwelling older adults in Europe. With the proportion of older adults worldwide continuing to rise, this issue requires urgent attention. Future research should focus on identifying the key factors preventing a decline in fall rates. Additionally, researchers and policymakers should work to overcome these barriers and improve the implementation of existing fall prevention strategies.

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Data Availability: Data available on request from the authors.

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